

Urology Clinical Guidelines

1. Hematuria Workup

Gross hematuria or painless gross hematuria in adults is a red flag and requires immediate evaluation to rule out malignancy. The standard evaluation includes a CT urography to assess the upper urinary tract and cystoscopy to assess the bladder and urethra. Microscopic hematuria also requires workup depending on patient risk factors.

2. BPH Evaluation

Evaluation of Benign Prostatic Hyperplasia (BPH) includes the IPSS scoring questionnaire, uroflowmetry, and post-void residual (PVR) measurement. First-line medical therapy includes alpha-blockers (e.g., tamsulosin) and 5-alpha reductase inhibitors (5-ARIs). Urinary retention is a severe complication of BPH.

3. Kidney Stone Management

Patients presenting with flank pain should be evaluated for nephrolithiasis. Non-contrast CT KUB is the imaging modality of choice. Management depends on stone size. Medical Expulsive Therapy (MET) may be used for small stones. Larger stones may require surgical intervention such as ureteroscopy or ESWL. Fever with flank pain suggests an infected stone and is a urologic emergency due to sepsis risk.

4. UTI Workup

Urinalysis and urine culture are standard for diagnosing UTI. Distinguish between complicated and uncomplicated UTIs.

5. Red Flags

Acute testicular pain requires immediate evaluation with scrotal ultrasound to rule out testicular torsion, a surgical emergency. Urinary retention and fever with flank pain also require urgent

evaluation.